BY ORDER OF THE COMMANDER
94TH AIRLIFT WING

94TH AIRLIFT WING INSTRUCTION 48-100

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Aerospace Medicine

EXPOSURE CONTROL PLAN FOR BLOODBORNE PATHOGENS

COMPLIANCE WITH THIS PUBLICATION IS MANDATORY

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This instruction implements the requirements of Occupational Safety & Health Administration (OSHA) Standard 29 CFR 1910.1030, *Occupational Exposure to Bloodborne Pathogens*.

This instruction applies to members assigned to the 94th Airlift Wing and Air Force tenant units. This written exposure control plan has been developed by the 94 MSG/SGPB (Bioenvironmental Engineering and Military Public Health Services) to eliminate or minimize employee exposure to blood or other potentially infectious materials. It provides guidance and procedures on encountering routine or non-routine occupational exposure to bloodborne pathogens including the HBVVirus (HBV) and Human Immunodeficiency Virus (HIV). Refer recommended changes and questions about this publication to the OPR using the AF FORM 847, *Recommendation for Change of Publication;* route AF IMTs 847 from the field through major command (MAJCOM) publications/forms managers. Ensure that all records created as a result of processes prescribed in this publication are maintained in accordance with Air Force Manual (AFMAN 33-363), *Management of Records,* and disposed of in accordance with the Air Force Records Disposition Schedule (RDS) located at **https://www.my.af.mil/afrims/afrims/afrims/rims.cfm**.

1. METHODS OF COMPLIANCE. 2
2. Personal Protective Equipment: 3
3. Housekeeping: 4
4. HEPATITIS B VACCINATION AND POST-EXPOSURE EVALUATION/FOLLOW UP: 4

5. POST EXPOSURE EVALUATION/FOLLOW-UP: 5
6. LABELING: 6
7. TRAINING: 6
8. RECORDKEEPING: 7
9. INFORMED CONSENT FOR Hep B Vaccination (RECOMBINANT): 8

Attachment 1—GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION 10

Attachment 2—EXPOSURE DETERMINATION 12

Attachment 3—HEPATITIS B VACCINE ACCEPTANCE 13

Attachment 4—TASKS WITH RECOMMENDED PERSONNEL PROTECTIVE EQUIPMENT 14

Attachment 5—EXPOSURE INCIDENT REPORT 15

Attachment 6—BLOODBORNE PATHOGENS TRAINING 16

Attachment 7—OSHA STANDARD: 29 CFR 1910.1030 17

1. METHODS OF COMPLIANCE.

1.1. Universal Precautions: Effective immediately, universal precautions shall be observed to prevent contact with blood or other potentially infectious materials. According to the concept of Universal Precautions, all human blood and certain human body fluids are treated as if known to be infectious for HIV, HBV, and other bloodborne pathogens. Under circumstances in which differentiation between body fluid types is difficult or impossible, all body fluids shall be considered potentially infectious materials. Supervisors of employees working in job classifications who encounter occupational exposure to blood or other potentially infectious materials are responsible for ensuring that employees observe universal precautions at all times.

1.2. Engineering and Work Practice Controls: Engineering and work practice controls shall be utilized as a primary method for eliminating or controlling exposure to blood or other potentially infectious materials. The following work practice controls will be utilized and enforced by department supervisors:

1.2.1. Employees MUST wash their hands and any other exposed skin with soap and water, or flush mucous membranes with water immediately or as soon as feasible following contact of such body areas with blood or other potentially infectious materials.

1.2.2. Employees MUST wash their hands and any other exposed skin immediately or as soon as possible after removal of gloves or other personal protective equipment.

1.2.3. Employees MUST wash their hands and any other exposed skin with soap and running water as soon as feasible after using an appropriate antiseptic. Hand cleaners or towelettes are acceptable only where hand washing facilities are not feasible.

1.2.4. Contaminated needles and other sharps shall not be bent, recapped, or removed unless no alternative is feasible or such action is required by specific medical procedures. Such recapping or needle removal must be accomplished through the use of a mechanical device or a one-handed technique. SHEARING OR BREAKING OF CONTAMINATED NEEDLES IS PROHIBITED.

1.2.5. Eating, drinking, smoking, applying cosmetics or lip balm, and handling contact lenses are prohibited in work areas where there is a reasonable likelihood of occupational exposure.

1.2.6. Food and drink shall not be kept in refrigerators, freezers, shelves, cabinets or on countertops or bench tops where blood or other potentially infectious materials are present.

1.2.7. All procedures involving blood or other potentially infectious materials shall be performed in such a manner as to minimize splashing, spraying, splattering, and generation of droplets of these substances. (See Attachment 4, Tasks with Recommended Personnel Protective Equipment)

1.2.8. Mouth pipetting/suctioning of blood or other potentially infectious materials is PROHIBITED.

1.2.9. Specimens of blood or other potentially infectious materials shall be placed in a container which prevents leakage during collection, handling, processing, storage, transport, or shipping.

1.2.10. Equipment which may become contaminated with blood or other potentially infectious materials shall be examined prior to servicing or shipping and decontaminated as necessary. If decontamination is not feasible, a readily observable label, in accordance with 29 CFR 1910.1030, must be attached to the equipment stating which portions remain contaminated. The Fire Department Infection Control Officer is responsible for informing affected 94 MSG/CEF employees, the servicing representative, and/or the manufacturer prior to handling, servicing, or shipping so that appropriate precautions can be taken.

2. Personal Protective Equipment: Where the risk occupational exposure remains after institution of engineering and work practice controls, appropriate personal protective equipment will be used. Personal protective equipment will be considered "appropriate" only if it does not permit blood or other potentially infectious materials to pass through to reach employee's work clothes, street clothes, undergarments, skin, eyes, mouth, or other mucous membranes under normal conditions of use. Personal protective equipment is provided at no cost to the employee. Supervisors will be responsible for ensuring that employees wear appropriate personal protective equipment. It is recommended that a First Aid Kit be place in each work area identified in the first table of “exposure determination.” The following also applies to personal protective equipment:

2.1. Personal protective equipment MUST be cleaned, laundered, repaired, and/or replaced as needed to maintain its effectiveness.

2.2. If a garment is penetrated by blood or other potentially infectious material, this garment MUST be removed immediately or as soon as feasible.

2.3. All personal protective equipment MUST be removed prior to leaving the work area.

2.4. When personal protective equipment is removed, it MUST be placed in an appropriately designated covered container for storage, washing and decontamination, or disposal.

3. Housekeeping: In keeping with the concept of Universal Precautions, supervisors will ensure that the worksite is maintained in a clean and sanitary condition.

3.1. Equipment: All contaminated equipment and environmental work surfaces shall be cleaned and decontaminated with an appropriate disinfectant after contact with blood or other potentially infectious materials by shift personnel.

3.2. Work Surfaces: Contaminated work surfaces shall be decontaminated with an appropriate disinfectant after completion of procedures or as soon as feasible when surfaces are obviously contaminated, after any spill of blood or other potentially infectious material, and at the end of the work shift.

3.3. Protective Coverings: Protective coverings such as plastic wrap, aluminum foil, or imperviously-backed absorbent paper used to cover equipment or environmental surfaces shall be removed and replaced as soon as feasible when they become obviously contaminated.

3.4. Trash Cans: All bins, pails, cans, and similar receptacles which have a reasonable likelihood for becoming contaminated with blood or other potentially infectious materials will be inspected, cleaned, and decontaminated as soon as feasible upon visible contamination.

3.5. Sharps: Contaminated sharps shall be discarded immediately or as soon as feasible in approved containers. CAUTION: Broken glassware which may be contaminated shall not be picked up directly with the hands. It must be cleaned up using mechanical means such as a brush and dust pan, tongs, or forceps. (Furthermore, any mechanical device which is contaminated must be decontaminated following use or as soon as feasible).

3.6. Sharps Containers: Sharps containers will be inspected to ensure they are not allowed to become overfilled. Sharps containers must be closable, puncture resistant, leak proof on sides and bottom, and labeled or color-coded in accordance with paragraph (g)(1)(i) of the standard. Additionally, sharps containers will be located as close as feasible to the immediate area where sharps are used.

3.7. Laundry: Contaminated laundry must be bagged or containerized at the location where it was used in an approved bag or container (see labeling requirements). Contaminated laundry must not be sorted or rinsed in the location of use. Contaminated laundry will be given to a contract laundry cleaning company with instructions to ensure disinfection.

3.8. Disposal: All contaminated waste, properly bagged and labeled, will be given to the 94 ASTS, located at Dobbins ARB, GA, within 12 hours of being generated. The organizations that should routinely generate "potentially infectious material" are the 94 MSG/CEF, the 94 AES, the 94 ASTS, and the 94 FSS/FSVL.

4. HEPATITIS B VACCINATION AND POST-EXPOSURE EVALUATION/FOLLOW UP: The Hep B Vaccination and vaccination series shall be made available, at no cost, to all employees identified. The remainder of the identified employees (see exposure determination), upon exposure, will be evaluated by a MD within 24 hours, who will make the decision for

treatment and follow-up, (to include hepatitis B immune globulin) at no cost to the employee. The 94 ASTS will provide the Hep B Vaccination to all reservists. The assigned contract medical facility will provide Hep B Vaccination to all civilians with occupational exposure. These employees will receive the Hep B Vaccination after the employee has received the training required (see Training) and within 10 working days of initial assignments. Civilian employees who decline the Hep B Vaccination will be required to sign the statement given in attachment 3. (ALL **RESERVISTS** NOW RECEIVE THE VACCINE). If a civilian employee initially declines the Hep B Vaccination but later decides to accept, 94 AW will make it available to the member, assuming the employee still has occupational exposure. Once the Hep B Vaccination series is completed booster doses are not recommended, nor is routine serologic testing to assess immune status of vaccines recommended. This will only be conducted by direction of a physician during post exposure evaluation and follow-up.

5. POST EXPOSURE EVALUATION/FOLLOW-UP: Due to the potentially severe consequences resulting in exposure incidents, the circumstances regarding these incidents will be investigated with the highest priority. Employees MUST notify their supervisor, within one hour of incident. The supervisor then notifies the 94 MSG/SGPB. The supervisor and employee complete "Exposure Incident Report," see attachment 5. The supervisor or the 94 MSG/SGPB then arranges, if necessary, for an evaluation by an MD within 24 hours. Post exposure evaluation and follow-ups are at no cost to the employee IAW the Bloodborne Pathogen Standard (29CFR1910.1030). If an exposure incident has occurred, the 94 AW/SE will be informed for proper documentation in the OSHA 300 log. The medical evaluation and follow-up will include the following elements:

5.1. Evaluation and documentation by an MD of the route(s) of exposure and the circumstances under which the exposure incident occurred. This should occur within 24 hours of exposure.

5.1.1. Personnel in civilian status at the time of exposure or their supervisor must contact the Air Force Personnel Center (AFPC) Injury Compensation Office at 1-800-525-0102 to request a CA-16, "Authorization for Examination and/or Medical Treatment" to be provided to the medical facility. Notification should also be made to the Civilian Personnel Office, the 94 FSS/FSMC (678) 655-4978.

5.1.2. Personnel in military status at the time of exposure will receive medical evaluation and follow-up through the 94 ASTS, (678) 655-4995.

5.2. Identification and documentation of the source individual, unless infeasible or prohibited by state or local law. If consent is obtained (where required), the source individual's blood shall be tested and the results documented. If the source individual is known to be infected with HIV or Hep B Vaccination, this shall be documented without a repeat test.

5.3. Results of the source individual's testing shall be made available to the exposed employee, along with applicable laws and regulations concerning disclosure of the identity and infectious status of the source individual.

5.4. The exposed employee's blood shall be tested as soon as feasible after consent is obtained.

5.5. If the employee consents to baseline blood collection but does not give consent at the time for HIV serologic testing, the sample shall be preserved for 90 days. If, within 90 days of the exposure incident, the employee elects to have the baseline sample tested, such testing shall be done as soon as feasible.

5.6. When medically indicated, post-exposure prophylaxis will be provided, as recommended by the U.S. Public Health Service.

5.7. Counseling will be made available to the employee upon request.

5.8. Evaluation of reported illnesses:

5.8.1. Within 15 days of completion, a copy of the evaluating healthcare professional's written opinion shall be obtained and provided to the employee. This written opinion will be limited to the following information:

5.8.1.1. That the employee has been informed of the results of the evaluation.

5.8.1.2. That the employee has been told about any medical conditions resulting from exposure to blood or other potentially infectious materials which require further evaluation or treatment (OTHER FINDINGS OR DIAGNOSES SHALL REMAIN CONFIDENTIAL AND NOT BE INCLUDED IN THE WRITTEN REPORT).

5.8.2. The 94 MSG/SGPB is responsible for providing the following information to the healthcare professional following an exposure incident and prior to medical evaluation:

5.8.2.1. A copy of 29 CFR 1910.1030 and the 94 AW ECP.

5.8.2.2. A description of the exposed employee's duties as they relate to the exposure incident.

5.8.2.3. Documentation of the route(s) of exposure and circumstances under which exposure occurred.

5.8.2.4. Results of the source individual's blood testing, if available.

5.8.2.5. All medical records relevant to the appropriate treatment of the employee including vaccination status.

6. LABELING: Warning labels shall be affixed to containers of regulated waste, refrigerators and freezers containing blood or other potentially infectious materials, and other containers used to store, transport, or ship blood or other potentially infectious materials. These labels shall include the following legend: **"BIOHAZARD" (word and symbol)**

6.1. These signs shall be fluorescent orange or orange-red or predominantly so, with lettering or symbols in contrasting color. Alternately, red bags or containers may be substituted for labels.

7. TRAINING: All employees with potential for occupational exposure will be expected to participate in a training session that will be provided at the time of initial assignment to tasks where potential occupational exposure takes place, every year thereafter, and whenever changes such as modifications of tasks or procedures or institution of new tasks or procedures affect the employee's exposure. The 94 MSG/SGPB will either provide training to all units or train a trainer (medical and firefighters will conduct their own.) Training will consist of the following:

7.1. An explanation of the Bloodborne Pathogens Standard (29 CFR 1910.1030) and the fact that a copy of the text of this standard will be accessible to employees at all times.

7.2. A general explanation of the epidemiology and symptoms of bloodborne diseases.

7.3. An explanation of the modes of transmission of bloodborne pathogens.

7.4. An explanation and of the exposure control plan and the means by which employees can obtain a copy of the written plan.

7.5. Inform the employee that a review of the exposure control plan is required annually, that is to reflect changes in technology that will help eliminate or reduce exposure to bloodborne pathogens. This review must include documentation of the consideration and implementation of appropriate commercially available and effective safer devices.

7.6. An explanation of the appropriate methods for recognizing tasks and other activities that may involve exposure to blood and other potentially infectious materials.

7.7. An explanation of the use and limitations of methods that will prevent or reduce exposure including engineering controls, work practice, and personal protective equipment.

7.8. Information on the types, proper use, location, removal, handling, decontamination, and disposal of personal protective equipment.

7.9. An explanation of the basis for selection of personal protective equipment.

7.10. Employees must be informed of the employer's requirement to solicit input from non-managerial heath care workers regarding the identification, evaluation and selection of effective engineering controls, including safer medical devices.

7.11. Information on the Hep B Vaccination and a statement that the vaccine will be offered free of charge, to those identified.

7.12. Information on the appropriate actions to take and persons to contact in an emergency involving blood or other potentially infectious materials.

7.13. An explanation of the procedures to follow if an exposure incident occurs, including the method of reporting the incident and the medical follow-up that will be made available.

7.14. Information on the post-exposure evaluation and follow-up that the employer is required to provide for the employee following an exposure incident.

7.15. An explanation of the signs and labels and/or color coding that is used in the facility.

7.16. An opportunity for interactive questions and answers with the person conducting the training session.

7.17. The organization will keep a record of the training on file and send a copy to the 94 MSG/SGPB.

8. RECORDKEEPING: Medical records must be kept for each employee with occupational exposure to Bloodborne Pathogens. Civilian and reserve medical records will be maintained IAW established procedures. In addition, the 94 MSG/SGPB will maintain a log for all exposures (file 31a.) These records will be kept IAW privacy act. The following records will be kept in the employee's medical record:

8.1. A file for each employee with occupational exposure to blood or other potentially infectious materials including the name and last four social security number of the employee, a copy of the employee's Hep B Vaccination status, any medical records relative to the employee's ability to receive vaccination.

8.2. A copy of all results of examinations, medical testing, and follow-up procedures following an exposure incident.

8.3. The employer's copy of the healthcare professional's written opinion regarding post-exposure evaluation and follow-up.

8.4. Hep B Vaccination declination statement of civilian workers will be filed in medical records.

8.5. The Public Health section of the Bioenvironmental Engineering office will develop and maintain a sharps injury log. It must be maintained in a way that ensures employee privacy and contain at a minimum, the following information: type and brand of device involved in the incident (if known), location of the incident; and description of the incident. ***NOTE:*** The above records will not be disclosed or reported without the employee's express written consent to any person within or outside the workplace except as required by the bloodborne pathogens standard or by law. Additionally, these records will be maintained for at least the duration of employment plus thirty (30) years.

9. INFORMED CONSENT FOR Hep B Vaccination (RECOMBINANT):

9.1. HBV Recombinant vaccine is a non-infectious viral vaccine that is provided at no charge to employees who are exposed or have the potential for occupational exposure to blood or body substance. It contains purified HBV Surface antigen (HBsAG) produced in genetically engineered yeast cells. The vaccine may contain up to 5% yeast protein. This recombinant vaccine is not made from human blood or blood products.

9.2. The vaccine is indicated for immunization of all health care workers and emergency response personnel at risk of infection from HBV virus as a result of occupational exposure to blood, blood products or other body substances.

9.3. HBV may occur when the virus, transmitted by infected bodily fluids is implanted on mucous membranes or introduced through breaks in the skin. HBV infection can have serious, life threatening consequences including acute liver failure, chronic hepatitis and cirrhosis of the liver.

9.4. The virus is predominantly in the blood of patients with active HBV or milk, urine, semen, and vaginal secretions. Transmission is also associated with close interpersonal contact with an infected individual and crowded living conditions.

9.5. The recombinant vaccine provides protective levels of antibodies in greater than 90% of healthy adults who receive all three doses. Long-term studies indicate that protection against HBV infection lasts for at least nine years following vaccination. The need for booster doses has not yet been defined.

9.6. The vaccine is contraindicated for those with a hypersensitivity to yeast, aluminum hydroxide or thimerosal (a mercury derivative).

9.7. Vaccination during pregnancy: Although it is always difficult to prove that adverse effects could not occur when any vaccine is given to pregnant women, the Center for Disease Control states the following: "On the basis of limited experience, there is no apparent risk of adverse effects to developing fetuses when HBV vaccine is administrated to pregnant women (CDC, Unpublished Data). The vaccine contains non- infectious HBsAG particles and should cause no risk to the fetus. Therefore, neither pregnancy nor lactation should be considered a contraindication to vaccination of women."

9.8. The vaccine is generally well tolerated. No serious adverse reactions were reported in clinical studies involving over 10,000 individuals. However, as with any vaccine, further broad use could reveal rare adverse reactions not previously noted. The most common reactions are local soreness, redness, and swelling at the injection site. Generalized complaints occur infrequently. They may include fatigue/ weakness, headache, dizziness, fever, rash, malaise, nausea, abdominal cramps, diarrhea and cold-like symptoms. Rare adverse experiences reported following vaccination, which may or may not be related to the vaccine, have included anaphylaxis, palpitations, asthma-like symptoms, neurologic symptoms and visual disturbances.

9.9. The vaccine is given intramuscularly in three doses. The first two doses are given one month apart, and the third dose six months after the first. Persons with immunodeficiency, on hemodialysis or those receiving immunosuppressive therapy (radiation therapy, chemotherapy, steroids), should report this information before receiving the vaccine. Also, those with any active infection or severe heart or lung condition should report this before receiving the vaccine.

9.10. Recipients of the vaccine should continue to use standard precautions in handling blood, blood products and other bodily fluids. They should also continue to report needle sticks and other body substance exposure to their supervisor.

BRETT J. CLARK, Colonel, USAF
Commander

Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

References

Occupational Safety & Health Administration (OSHA) Standard 29 CFR 1910.1030, *Occupational Exposure to Bloodborne Pathogens,*

Abbreviations and Acronyms

HBV—hepatitis B virus

HBIG—HBV immune globulin is a blood plasma product that can prevent hepatitis B if given within 14 days of an exposure to an infected individual.

HIV—human immunodeficiency virus

MD—Doctor of Medicine

OSHA—Occupational Safety & Health Administration

Terms

Blood— Human blood, human blood components, and products made from human blood.

Bloodborne Pathogens—Pathogenic microorganisms that are present in human blood and can cause disease in humans. These pathogens include, but are not limited to, hepatitis B virus (Hep B Vaccination) and human immunodeficiency virus (HIV).

Contaminated—The presence or the reasonably anticipated presence of blood or other potentially infectious material on an item or surface.

Contaminated Laundry—Laundry which has been soiled with blood or other potentially infectious materials or may contain sharps.

Contaminated Sharps—Any contaminated object that can penetrate the skin including, but not limited to, needles, scalpels, broken glass, broken capillary tubes and exposed ends of dental wires.

Decontamination—The use of physical or chemical means to remove, inactivate, or destroy bloodborne pathogens on a surface or item to the point where they are no longer capable of transmitting infectious articles and the surface or item is rendered safe for handling, use or disposal.

Engineering Controls—Controls (e.g., sharps disposal containers, self-sheathing needles, safer medical devices, such as sharps with engineered sharps injury protections and needleless systems) that isolate or remove the bloodborne pathogens hazard from the workplace.

Exposure Incident—A specific eye, mouth, other mucous membrane, non-intact skin, or parenteral contact with blood or other potentially infectious materials that results from the performance of an employee's duties.

Needleless Systems—A device that does not use needles for: (1) The collection of bodily fluids or withdrawal of body fluids after initial venous or arterial access is established; (2) The administration of medication or fluids; or (3) Any other procedure involving the potential for

occupational exposure to bloodborne pathogens due to percutaneous injuries from contaminated sharps.

Occupational Exposure—Reasonably anticipated skin, eye, mucous membrane, or parenteral contact with blood or other potentially infectious materials that results from the performance of an employee's duties.

Other Potentially Infectious Materials—(1) The following human body fluids: semen, vaginal secretions, cerebrospinal fluid, synovial fluid, pleural fluid, pericardial fluid, peritoneal fluid, amniotic fluid, saliva in dental procedures, any body fluid that is visibly contaminated with blood, and all body fluids in situations where it is difficult or impossible to differentiate between body fluids; (2) Any unfixed tissue or organ (other than intact skin) from a human (living or dead); and (3) HIV-containing cell or tissue cultures, organ cultures, and HIV or Hep B Vaccination-containing culture medium or other solutions; and blood, organs, or other tissue from experimental animals infected with HIV or Hep B Vaccination.

Parenteral—Piercing mucous membranes or the skin barrier through such events as needle stick, human bites, cuts, and abrasions.

Personal Protective Equipment—Specialized clothing or equipment worn by an employee for protection against a hazard. General work clothes (e.g. uniforms, pants, shirts, or blouses) not intended to function as protection against a hazard is not considered to be personal protective equipment.

Sharps with engineered sharps injury protections—a nonneedle sharp or a needle device used for withdrawing body fluids, accessing a vein or artery, or administering medications or other fluids, with a built-in safety feature or mechanism that effectively reduces the risk of an exposure incident.

Regulated Waste—Liquid or semi-liquid blood or other potentially infectious materials; contaminated items that would release blood or other potentially infectious materials in a liquid or semi-liquid state if compressed; items that are caked with dried blood or other potentially infectious materials and are capable of releasing these materials during handling; contaminated sharps; and pathological and microbiological wastes containing blood or other potentially infectious materials.

Universal Precautions—An approach to infection control. According to the concept of Universal Precautions, all human blood and certain human body fluids are treated as if known to be infectious for HIV, Hep B Vaccination, and other bloodborne pathogens.

Work Practice Controls—Controls that reduce the likelihood of exposure by altering the manner in which a task is performed (e.g. prohibiting recapping of needles by a two handed technique).

Attachment 2

EXPOSURE DETERMINATION

Exposure determination has been made without regard to the use of personal protective equipment. The following are job classifications in which employees have **potential** exposure to bloodborne pathogens:

UNIT	WHO?	Hep B Vaccination *	RATIONALE	Task #
94 MSG/CEF	Fire Fighters	OFFERED	EMERGENCY RESPONSE, CONFINED SPACE RESCUE	2 & 4-14
94 CES/CEF	Fire Fighters	REQUIRED	EMERGENCY RESPONSE	2 & 4-14
94 ASTS	All reservists	REQUIRED	RESERVE MEDICAL	1-13
94 AES	All reservists who provide patient care.	REQUIRED	RESERVE MEDICAL	1-13
94 MXS/MXMCF	Fuel Cell Workers & Fuel Cell Attendants	OFFERED (CIV) REQUIRED (MIL)	CONFINED SPACE ENTRY/FUEL CELL PLAN	6, 12
94 SFS	Security Police Officers	OFFERED (CIV) REQUIRED (MIL)	EMERGENCY RESPONSE, VEHICLE SEARCHES, UNCOOPERATIVE INDIVIDUAL	6, 13
94 FSS/SVH	Lodging lead housekeepers	OFFERED	MATERIALS IN LODGING ROOMS CONTAMINATED WITH BODY FLUID (Reference Lodging OI)	13
94 FSS Mortuary Affairs	Search and Recovery Team FSS Maintains List	OFFERED (CIV) REQUIRED (MIL)	SEARCH AND RECOVERY OPERATIONS	15
94 FSS/SVR	Fitness Center Staff	OFFERED (CIV)	WIPING OFF EQUIPMENT AND/OR RENDERING AID	6, 7, 8, 11, 13, 14

Attachment 3

HEPATITIS B VACCINE ACCEPTANCE

I have read the above facts about Recombinant HBVvaccine and I have had the opportunity to ask questions. I request that the vaccine be given to me.

PRINT NAME _____

SIGNATURE _____

LAST FOUR SSN _____JOB TITLE _____

UNIT _____PHONE _____DATE _____

DECLINATION STATEMENT FOR HEPATITIS B IMMUNIZATION

I understand that due to my occupational exposure to blood or other potentially infectious materials, I may be at-risk of acquiring hepatitis B virus (Hep B Vaccination) infection. I have been given the opportunity to be vaccinated with the HBVvaccine, at no charge to myself.

However, I decline HBVvaccination at this time. I understand that by declining this vaccine, I continue to be at-risk of acquiring Hepatitis B, a serious disease. If in the future, I continue to have occupational exposure to blood and other potentially infectious materials while employed by the Department of Defense and I want to be vaccinated with HBVvaccine, I can receive the vaccination series at no charge to me.

PRINT NAME _____

SIGNATURE _____

LAST FOUR SSN _____JOB TITLE _____

UNIT _____PHONE _____DATE _____

WITNESS _____

Attachment 4

TASKS WITH RECOMMENDED PERSONNEL PROTECTIVE EQUIPMENT

#	Task	Gloves	Eye Protection	Masks	Gown
1	**Drawing blood**	Yes	No	No	No
2	**Starting IV**	Yes	No	No	No
3	**Giving injection**	Yes	No	No	No
4	**Blood pressure**	No	No	No	No
5	**Temperature**	No	No	No	No
6	**Direct patient contact**	Yes	No	No	No
7	**Bleeding control with minimal bleeding**	Yes	No	No	No
8	**Bleeding control with spurting blood**	Yes	Yes	Yes	Yes
9	**Suctioning**	Yes	No, unless splashing	No, unless splashing	No
10	**Childbirth**	Yes	Yes	Yes	Yes
11	**Disinfecting Contaminated Equipment**	Yes	Yes	No	Yes
12	**Administering CPR**	Yes	No	Mouth to mouth mask with one way valve	No
13	**Contact with contaminated equipment**	Yes	No	No	No
14	**Utilize AED (Automatic External Defibrillator)**	Yes	No	No	No
15	**Locate, mark area & and retrieve causalities from crash site**	Yes	No	Yes	Yes

Attachment 5

EXPOSURE INCIDENT REPORT

EMPLOYEE NAME _____

DATE OF BIRTH _____ SSN# _____

TELEPHONE (W) _____ (H) _____

JOB TITLE _____

DATE OF EXPOSURE _____ TIME _____

HEPATITIS B VACCINATION STATUS _____

LOCATION OF INCIDENT _____

DESCRIBE WHAT JOB DUTIES YOU WERE PERFORMING WHEN THE EXPOSURE INCIDENT OCCURRED:

_____
_____
_____

WHAT BODY FLUID(S) WERE YOU EXPOSED TO? _____
_____

WHAT WAS THE ROUTE OF EXPOSURE? (MUCOSAL CONTACT, CONTACT WITH NON-INTACT SKIN, PERCUTANEOUS)

_____
_____

DESCRIBE ANY PERSONAL PROTECTIVE EQUIPMENT IN USE AT THE TIME OF EXPOSURE INCIDENT:

_____

DID PPE FAIL? _____ IF YES, HOW? _____
_____

SOURCE INDIVIDUAL'S NAME:
_____

OTHER INFORMATION, SUCH AS SEEN BY MD: _____
_____

Attachment 6

BLOODBORNE PATHOGENS TRAINING

TRAINER	
TRAINER'S QUALIFICATIONS	
DATE OF TRAINING	
UNIT	
SUMMARY OF TRAINING SESSION:	

NAME OF ATTENDEES	LAST FOUR SSN	JOB TITLE

Attachment 7

OSHA STANDARD: 29 CFR 1910.1030

Standard Title: Bloodborne pathogens
SubPart Number: Z
SubPart Title: Toxic and Hazardous Substances

(a)Scope and Application. This section applies to all occupational exposure to blood or other potentially infectious materials as defined by paragraph (b) of this section.

(b)Definitions. For purposes of this section, the following shall apply:

Assistant Secretary means the Assistant Secretary of Labor for Occupational Safety and Health, or designated representative.

Blood means human blood, human blood components, and products made from human blood.

Bloodborne Pathogens means pathogenic microorganisms that are present in human blood and can cause disease in humans. These pathogens include, but are not limited to, hepatitis B virus (Hep B Vaccination) and human immunodeficiency virus (HIV).

Clinical Laboratory means a workplace where diagnostic or other screening procedures are performed on blood or other potentially infectious materials.

Contaminated means the presence or the reasonably anticipated presence of blood or other potentially infectious materials on an item or surface.

Contaminated Laundry means laundry which has been soiled with blood or other potentially infectious materials or may contain sharps.

Contaminated Sharps means any contaminated object that can penetrate the skin including, but not limited to, needles, scalpels, broken glass, broken capillary tubes, and exposed ends of dental wires.

Decontamination means the use of physical or chemical means to remove, inactivate, or destroy bloodborne pathogens on a surface or item to the point where they are no longer capable of transmitting infectious particles and the surface or item is rendered safe for handling, use, or disposal.

Director means the Director of the National Institute for Occupational Safety and Health, U.S. Department of Health and Human Services, or designated representative.

Engineering Controls means controls (e.g., sharps disposal containers, self-sheathing needles, safer medical devices, such as sharps with engineered sharps injury protections and needleless systems) that isolate or remove the bloodborne pathogens hazard from the workplace.

Exposure Incident means a specific eye, mouth, other mucous membrane, non-intact skin, or parenteral contact with blood or other potentially infectious materials that results from the performance of an employee's duties.

Handwashing Facilities means a facility providing an adequate supply of running potable water, soap and single use towels or hot air drying machines.

Licensed Healthcare Professional is a person whose legally permitted scope of practice allows him or her to independently perform the activities required by paragraph (f) HBVVaccination and Post-exposure Evaluation and Follow-up.

Hep B Vaccination means hepatitis B virus.

HIV means human immunodeficiency virus.

Needleless systems means a device that does not use needles for: (1) The collection of bodily fluids or withdrawal of body fluids after initial venous or arterial access is established; (2) The administration of medication or fluids; or (3) Any other procedure involving the potential for occupational exposure to bloodborne pathogens due to Percutaneous injuries from contaminated sharps.

Occupational Exposure means reasonably anticipated skin, eye, mucous membrane, or parenteral contact with blood or other potentially infectious materials that may result from the performance of an employee's duties.

Other Potentially Infectious Materials means (1) The following human body fluids: semen, vaginal secretions, cerebrospinal fluid, synovial fluid, pleural fluid, pericardial fluid, peritoneal fluid, amniotic fluid, saliva in dental procedures, any body fluid that is visibly contaminated with blood, and all body fluids in situations where it is difficult or impossible to differentiate between body fluids; (2) Any unfixed tissue or organ (other than intact skin) from a human (living or dead); and (3) HIV-containing cell or tissue cultures, organ cultures, and HIV- or Hep B Vaccination-containing culture medium or other solutions; and blood, organs, or other tissues from experimental animals infected with HIV or Hep B Vaccination.

Parenteral means piercing mucous membranes or the skin barrier through such events as needlesticks, human bites, cuts, and abrasions.

Personal Protective Equipment is specialized clothing or equipment worn by an employee for protection against a hazard. General work clothes (e.g., uniforms, pants, shirts or blouses) not intended to function as protection against a hazard is not considered to be personal protective equipment.

Production Facility means a facility engaged in industrial-scale, large-volume or high concentration production of HIV or Hep B Vaccination. Regulated Waste means liquid or semi-liquid blood or other potentially infectious materials; contaminated items that would release blood or other potentially infectious materials in a liquid or semi-liquid state if compressed;

items that are caked with dried blood or other potentially infectious materials and are capable of releasing these materials during handling; contaminated sharps; and pathological and microbiological wastes containing blood or other potentially infectious materials.

Research Laboratory means a laboratory producing or using research-laboratory-scale amounts of HIV or Hep B Vaccination. Research laboratories may produce high concentrations of HIV or Hep B Vaccination but not in the volume found in production facilities.

Sharps with engineered sharps injury protections means a nonneedle sharp or a needle device used for withdrawing body fluids, accessing a vein or artery, or administering medications or other fluids, with a built-in safety feature or mechanism that effectively reduces the risk of an exposure incident.

Source Individual means any individual, living or dead, whose blood or other potentially infectious materials may be a source of occupational exposure to the employee. Examples include, but are not limited to, hospital and clinic patients; clients in institutions for the developmentally disabled; trauma victims; clients of drug and alcohol treatment facilities; residents of hospices and nursing homes; human remains; and individuals who donate or sell blood or blood components. Sterilize means the use of a physical or chemical procedure to destroy all microbial life including highly resistant bacterial endospores.

Universal Precautions is an approach to infection control. According to the concept of Universal Precautions, all human blood and certain human body fluids are treated as if known to be infectious for HIV, Hep B Vaccination, and other bloodborne pathogens.

Work Practice Controls means controls that reduce the likelihood of exposure by altering the manner in which a task is performed (e.g., prohibiting recapping of needles by a two-handed technique).

(c) Exposure Control --

(c)(1) Exposure Control Plan.

(c)(1)(i) Each employer having an employee(s) with occupational exposure as defined by paragraph (b) of this section shall establish a written Exposure Control Plan designed to eliminate or minimize employee exposure.

(c)(1)(ii) The Exposure Control Plan shall contain at least the following elements:

(c)(1)(ii)(A) The exposure determination required by paragraph (c) (2),

1910.1030(c) (1) (ii) (B)

(c)(1)(ii)(B) The schedule and method of implementation for paragraphs (d) Methods of Compliance, (e) HIV and Hep B Vaccination Research Laboratories and Production Facilities,

(f) HBVVaccination and Post-Exposure Evaluation and Follow-up, (g) Communication of Hazards to Employees, and (h) Recordkeeping, of this standard, and
(c)(1)(ii)(C) The procedure for the evaluation of circumstances surrounding exposure incidents as required by paragraph (f) (3) (i) of this standard.

(c)(1)(iii) Each employer shall ensure that a copy of the Exposure Control Plan is accessible to employees in accordance with 29 CFR 1910.1020(e).

(c)(1)(iv) The Exposure Control Plan shall be reviewed and updated at least annually and whenever necessary to reflect new or modified tasks and procedures which affect occupational exposure and to reflect new or revised employee positions with occupational exposure. The review and update of such plans shall also:

(c)(1)(iv)(A) Reflect changes in technology that eliminate or reduce exposure to bloodborne pathogens; and

(c)(1)(iv)(B) Document annually consideration and implementation of appropriate commercially available and effective safer medical devices designed to eliminate or minimize occupational exposure.

(c)(1)(v) An employer, who is required to establish an Exposure Control Plan shall solicit input from non-managerial employees responsible for direct patient care who are potentially exposed to injuries from contaminated sharps in the identification, evaluation, and selection of effective engineering and work practice controls and shall document the solicitation in the Exposure Control Plan.

(c)(2) Exposure Determination.

(c)(2)(i) Each employer who has an employee(s) with occupational exposure as defined by paragraph (b) of this section shall prepare an exposure determination. This exposure determination shall contain the following:

(c)(2)(i)(A) A list of all job classifications in which all employees in those job classifications have occupational exposure;

1910.1030(c)(2)(i)(B)

(c)(2)(i)(B) A list of job classifications in which some employees have occupational exposure, and

(c)(2)(i)(C) A list of all tasks and procedures or groups of closely related task and procedures in which occupational exposure occurs and that are performed by employees in job classifications listed in accordance with the provisions of paragraph (c)(2)(i)(B) of this standard.

(c)(2)(ii) This exposure determination shall be made without regard to the use of personal protective equipment.

(d) Methods of Compliance --

(d)(1) General. Universal precautions shall be observed to prevent contact with blood or other potentially infectious materials. Under circumstances in which differentiation between body fluid types is difficult or impossible, all body fluids shall be considered potentially infectious materials.

(d)(2) Engineering and Work Practice Controls.

(d)(2)(i) Engineering and work practice controls shall be used to eliminate or minimize employee exposure. Where occupational exposure remains after institution of these controls, personal protective equipment shall also be used.

1910.1030(d)(2)(ii)

(d)(2)(ii) Engineering controls shall be examined and maintained or replaced on a regular schedule to ensure their effectiveness.

(d)(2)(iii) Employers shall provide handwashing facilities which are readily accessible to employees.

(d)(2)(iv) When provision of handwashing facilities is not feasible, the employer shall provide either an appropriate antiseptic hand cleanser in conjunction with clean cloth/paper towels or antiseptic towelettes. When antiseptic hand cleansers or towelettes are used, hands shall be washed with soap and running water as soon as feasible.

(d)(2)(v) Employers shall ensure that employees wash their hands immediately or as soon as feasible after removal of gloves or other personal protective equipment.

(d)(2)(vi) Employers shall ensure that employees wash hands and any other skin with soap and water, or flush mucous membranes with water immediately or as soon as feasible following contact of such body areas with blood or other potentially infectious materials.

(d)(2)(vii) Contaminated needles and other contaminated sharps shall not be bent, recapped, or removed except as noted in paragraphs (d)(2)(vii)(A) and (d)(2)(vii)(B) below. Shearing or breaking of contaminated needles is prohibited.

1910.1030(d)(2)(vii)(A)

(d)(2)(vii)(A) Contaminated needles and other contaminated sharps shall not be bent, recapped or removed unless the employer can demonstrate that no alternative is feasible or that such action is required by a specific medical or dental procedure.

(d)(2)(vii)(B) Such bending, recapping or needle removal must be accomplished through the use of a mechanical device or a one-handed technique.

(d)(2)(viii) Immediately or as soon as possible after use, contaminated reusable sharps shall be placed in appropriate containers until properly reprocessed. These containers shall be:

(d)(2)(viii)(A) Puncture resistant;

(d)(2)(viii)(B) Labeled or color-coded in accordance with this standard;

(d)(2)(viii)(C) Leakproof on the sides and bottom; and

(d)(2)(viii)(D) In accordance with the requirements set forth in paragraph (d)(4)(ii)(E) for reusable sharps.

(d)(2)(ix) Eating, drinking, smoking, applying cosmetics or lip balm, and handling contact lenses are prohibited in work areas where there is a reasonable likelihood of occupational exposure.

(d)(2)(x) Food and drink shall not be kept in refrigerators, freezers, shelves, cabinets or on countertops or benchtops where blood or other potentially infectious materials are present.

1910.1030(d)(2)(xi)

(d)(2)(xi) All procedures involving blood or other potentially infectious materials shall be performed in such a manner as to minimize splashing, spraying, spattering, and generation of droplets of these substances.

(d)(2)(xii) Mouth pipetting/suctioning of blood or other potentially infectious materials is prohibited.

(d)(2)(xiii) Specimens of blood or other potentially infectious materials shall be placed in a container which prevents leakage during collection, handling, processing, storage, transport, or shipping.

(d)(2)(xiii)(A) The container for storage, transport, or shipping shall be labeled or color-coded according to paragraph (g)(1)(i) and closed prior to being stored, transported, or shipped. When a facility utilizes Universal Precautions in the handling of all specimens, the labeling/color-coding of specimens is not necessary provided containers are recognizable as containing specimens. This exemption only applies while such specimens/containers remain within the facility. Labeling or color-coding in accordance with paragraph (g)(1)(i) is required when such specimens/containers leave the facility.

(d)(2)(xiii)(B) If outside contamination of the primary container occurs, the primary container shall be placed within a second container which prevents leakage during handling, processing, storage, transport, or shipping and is labeled or color-coded according to the requirements of this standard.

1910.1030(d)(2)(xiii)(C)

(d)(2)(xiii)(C) If the specimen could puncture the primary container, the primary container shall be placed within a secondary container which is puncture-resistant in addition to the above characteristics.

(d)(2)(xiv) Equipment which may become contaminated with blood or other potentially infectious materials shall be examined prior to servicing or shipping and shall be decontaminated as necessary, unless the employer can demonstrate that decontamination of such equipment or portions of such equipment is not feasible.

(d)(2)(xiv)(A) A readily observable label in accordance with paragraph (g)(1)(i)(H) shall be attached to the equipment stating which portions remain contaminated.

(d)(2)(xiv)(B) The employer shall ensure that this information is conveyed to all affected employees, the servicing representative, and/or the manufacturer, as appropriate, prior to handling, servicing, or shipping so that appropriate precautions will be taken.

(d)(3) Personal Protective Equipment --

(d)(3)(i) Provision. When there is occupational exposure, the employer shall provide, at no cost to the employee, appropriate personal protective equipment such as, but not limited to, gloves, gowns, laboratory coats, face shields or masks and eye protection, and mouthpieces, resuscitation bags, pocket masks, or other ventilation devices. Personal protective equipment will be considered "appropriate" only if it does not permit blood or other potentially infectious materials to pass through to or reach the employee's work clothes, street clothes, undergarments, skin, eyes, mouth, or other mucous membranes under normal conditions of use and for the duration of time which the protective equipment will be used.

(d)(3)(ii) Use. The employer shall ensure that the employee uses appropriate personal protective equipment unless the employer shows that the employee temporarily and briefly declined to use personal protective equipment when, under rare and extraordinary circumstances, it was the employee's professional judgment that in the specific instance its use would have prevented the delivery of health care or public safety services or would have posed an increased hazard to the safety of the worker or co-worker. When the employee makes this judgment, the circumstances shall be investigated and documented in order to determine whether changes can be instituted to prevent such occurrences in the future.

(d)(3)(iii) Accessibility. The employer shall ensure that appropriate personal protective equipment in the appropriate sizes is readily accessible at the worksite or is issued to employees. Hypoallergenic gloves, glove liners, powderless gloves, or other similar alternatives shall be readily accessible to those employees who are allergic to the gloves normally provided.

(d)(3)(iv) Cleaning, Laundering, and Disposal. The employer shall clean, launder, and dispose of personal protective equipment required by paragraphs (d) and (e) of this standard, at no cost to the employee.

1910.1030(d)(3)(v)

(d)(3)(v) Repair and Replacement. The employer shall repair or replace personal protective equipment as needed to maintain its effectiveness, at no cost to the employee.

(d)(3)(vi) If a garment(s) is penetrated by blood or other potentially infectious materials, the garment(s) shall be removed immediately or as soon as feasible.

(d)(3)(vii) All personal protective equipment shall be removed prior to leaving the work area.

(d)(3)(viii) When personal protective equipment is removed it shall be placed in an appropriately designated area or container for storage, washing, decontamination or disposal.

(d)(3)(ix) Gloves. Gloves shall be worn when it can be reasonably anticipated that the employee may have hand contact with blood, other potentially infectious materials, mucous membranes, and non-intact skin; when performing vascular access procedures except as specified in paragraph (d)(3)(ix)(D); and when handling or touching contaminated items or surfaces.

(d)(3)(ix)(A) Disposable (single use) gloves such as surgical or examination gloves, shall be replaced as soon as practical when contaminated or as soon as feasible if they are torn, punctured, or when their ability to function as a barrier is compromised.

1910.1030(d)(3)(ix)(B)

(d)(3)(ix)(B) Disposable (single use) gloves shall not be washed or decontaminated for re-use.

(d)(3)(ix)(C) Utility gloves may be decontaminated for re-use if the integrity of the glove is not compromised. However, they must be discarded if they are cracked, peeling, torn, punctured, or exhibit other signs of deterioration or when their ability to function as a barrier is compromised.

(d)(3)(ix)(D) If an employer in a volunteer blood donation center judges that routine gloving for all phlebotomies is not necessary then the employer shall:

(d)(3)(ix)(D)(1) Periodically reevaluate this policy;

(d)(3)(ix)(D)(2) Make gloves available to all employees who wish to use them for phlebotomy;

(d)(3)(ix)(D)(3) Not discourage the use of gloves for phlebotomy; and

(d)(3)(ix)(D)(4) Require that gloves be used for phlebotomy in the following circumstances:

(d)(3)(ix)(D)(4)(i) When the employee has cuts, scratches, or other breaks in his or her skin;

(d)(3)(ix)(D)(4)(ii) When the employee judges that hand contamination with blood may occur, for example, when performing phlebotomy on an uncooperative source individual; and

(d)(3)(ix)(D)(4)(iii) When the employee is receiving training in phlebotomy.

1910.1030(d)(3)(x)

(d)(3)(x) Masks, Eye Protection, and Face Shields. Masks in combination with eye protection devices, such as goggles or glasses with solid side shields, or chin-length face shields, shall be worn whenever splashes, spray, spatter, or droplets of blood or other potentially infectious materials may be generated and eye, nose, or mouth contamination can be reasonably anticipated.

(d)(3)(xi) Gowns, Aprons, and Other Protective Body Clothing. Appropriate protective clothing such as, but not limited to, gowns, aprons, lab coats, clinic jackets, or similar outer garments shall be worn in occupational exposure situations. The type and characteristics will depend upon the task and degree of exposure anticipated.

(d)(3)(xii) Surgical caps or hoods and/or shoe covers or boots shall be worn in instances when gross contamination can reasonably be anticipated (e.g., autopsies, orthopedic surgery).

(d)(4) Housekeeping --

(d)(4)(i) General. Employers shall ensure that the worksite is maintained in a clean and sanitary condition. The employer shall determine and implement an appropriate written schedule for cleaning and method of decontamination based upon the location within the facility, type of surface to be cleaned, type of soil present, and tasks or procedures being performed in the area.

(d)(4)(ii) All equipment and environmental and working surfaces shall be cleaned and decontaminated after contact with blood or other potentially infectious materials.

1910.1030(d)(4)(ii)(A)

(d)(4)(ii)(A) Contaminated work surfaces shall be decontaminated with an appropriate disinfectant after completion of procedures; immediately or as soon as feasible when surfaces are overtly contaminated or after any spill of blood or other potentially infectious materials; and at the end of the work shift if the surface may have become contaminated since the last cleaning.

(d)(4)(ii)(B) Protective coverings, such as plastic wrap, aluminum foil, or imperviously-backed absorbent paper used to cover equipment and environmental surfaces, shall be removed and replaced as soon as feasible when they become overtly contaminated or at the end of the workshift if they may have become contaminated during the shift.

(d)(4)(ii)(C) All bins, pails, cans, and similar receptacles intended for reuse which have a reasonable likelihood for becoming contaminated with blood or other potentially infectious materials shall be inspected and decontaminated on a regularly scheduled basis and cleaned and decontaminated immediately or as soon as feasible upon visible contamination.

(d)(4)(ii)(D) Broken glassware which may be contaminated shall not be picked up directly with the hands. It shall be cleaned up using mechanical means, such as a brush and dust pan, tongs, or forceps.

(d)(4)(ii)(E) Reusable sharps that are contaminated with blood or other potentially infectious materials shall not be stored or processed in a manner that requires employees to reach by hand into the containers where these sharps have been placed.

(d)(4)(iii) Regulated Waste --

1910.1030(d)(4)(iii)(A)

(d)(4)(iii)(A) Contaminated Sharps Discarding and Containment.

(d)(4)(iii)(A)(1) Contaminated sharps shall be discarded immediately or as soon as feasible in containers that are:

(d)(4)(iii)(A)(1)(i) Closable;

(d)(4)(iii)(A)(1)(ii) Puncture resistant;

(d)(4)(iii)(A)(1)(iii) Leakproof on sides and bottom; and

(d)(4)(iii)(A)(1)(iv) Labeled or color-coded in accordance with paragraph (g)(1)(i) of this standard.

(d)(4)(iii)(A)(2) During use, containers for contaminated sharps shall be:

(d)(4)(iii)(A)(2)(i) Easily accessible to personnel and located as close as is feasible to the immediate area where sharps are used or can be reasonably anticipated to be found (e.g., laundries);

(d)(4)(iii)(A)(2)(ii) Maintained upright throughout use; and

(d)(4)(iii)(A)(2)(iii) Replaced routinely and not be allowed to overfill.

(d)(4)(iii)(A)(3) When moving containers of contaminated sharps from the area of use, the containers shall be:

(d)(4)(iii)(A)(3)(i) Closed immediately prior to removal or replacement to prevent spillage or protrusion of contents during handling, storage, transport, or shipping;

(d)(4)(iii)(A)(3)(ii) Placed in a secondary container if leakage is possible. The second container shall be:

(d)(4)(iii)(A)(3)(ii)(A) Closable;

(d)(4)(iii)(A)(3)(ii)(B) Constructed to contain all contents and prevent leakage during handling, storage, transport, or shipping; and

(d)(4)(iii)(A)(3)(ii)(C) Labeled or color-coded according to paragraph (g)(1)(i) of this standard.

(d)(4)(iii)(A)(4) Reusable containers shall not be opened, emptied, or cleaned manually or in any other manner which would expose employees to the risk of percutaneous injury.

(d)(4)(iii)(B) Other Regulated Waste Containment --

(d)(4)(iii)(B)(1) Regulated waste shall be placed in containers which are:

(d)(4)(iii)(B)(1)(i) Closable;

(d)(4)(iii)(B)(1)(ii) Constructed to contain all contents and prevent leakage of fluids during handling, storage, transport or shipping;

(d)(4)(iii)(B)(1)(iii) Labeled or color-coded in accordance with paragraph (g)(1)(i) this standard; and
(d)(4)(iii)(B)(1)(iv) Closed prior to removal to prevent spillage or protrusion of contents during handling, storage, transport, or shipping.

(d)(4)(iii)(B)(2) If outside contamination of the regulated waste container occurs, it shall be placed in a second container. The second container shall be:

(d)(4)(iii)(B)(2)(i) Closable;

(d)(4)(iii)(B)(2)(ii) Constructed to contain all contents and prevent leakage of fluids during handling, storage, transport or shipping;

(d)(4)(iii)(B)(2)(iii) Labeled or color-coded in accordance with paragraph (g)(1)(i) of this standard; and

(d)(4)(iii)(B)(2)(iv) Closed prior to removal to prevent spillage or protrusion of contents during handling, storage, transport, or shipping.

(d)(4)(iii)(C) Disposal of all regulated waste shall be in accordance with applicable regulations of the United States, States and Territories, and political subdivisions of States and Territories.

1910.1030(d)(4)(iv)

(d)(4)(iv) Laundry.

(d)(4)(iv)(A) Contaminated laundry shall be handled as little as possible with a minimum of agitation.

(d)(4)(iv)(A)(1) Contaminated laundry shall be bagged or containerized at the location where it was used and shall not be sorted or rinsed in the location of use.

(d)(4)(iv)(A)(2) Contaminated laundry shall be placed and transported in bags or containers labeled or color-coded in accordance with paragraph (g)(1)(i) of this standard. When a facility utilizes Universal Precautions in the handling of all soiled laundry, alternative labeling or color-coding is sufficient if it permits all employees to recognize the containers as requiring compliance with Universal Precautions.

(d)(4)(iv)(A)(3) Whenever contaminated laundry is wet and presents a reasonable likelihood of soak-through of or leakage from the bag or container, the laundry shall be placed and transported in bags or containers which prevent soak-through and/or leakage of fluids to the exterior.

(d)(4)(iv)(B) The employer shall ensure that employees who have contact with contaminated laundry wear protective gloves and other appropriate personal protective equipment.

1910.1030(d)(4)(iv)(C)

(d)(4)(iv)(C) When a facility ships contaminated laundry off-site to a second facility which does not utilize Universal Precautions in the handling of all laundry, the facility generating the contaminated laundry must place such laundry in bags or containers which are labeled or color-coded in accordance with paragraph (g)(1)(i).

(e) HIV and Hep B Vaccination Research Laboratories and Production Facilities.

(e)(1) This paragraph applies to research laboratories and production facilities engaged in the culture, production, concentration, experimentation, and manipulation of HIV and Hep B Vaccination. It does not apply to clinical or diagnostic laboratories engaged solely in the analysis of blood, tissues, or organs. These requirements apply in addition to the other requirements of the standard.

(e)(2) Research laboratories and production facilities shall meet the following criteria:

(e)(2)(i) Standard Microbiological Practices. All regulated waste shall either be incinerated or decontaminated by a method such as autoclaving known to effectively destroy bloodborne pathogens.

(e)(2)(ii) Special Practices.

(e)(2)(ii)(A)Laboratory doors shall be kept closed when work involving HIV or Hep B Vaccination is in progress.

1910.1030(e)(2)(ii)(B)

(e)(2)(ii)(B) Contaminated materials that are to be decontaminated at a site away from the work area shall be placed in a durable, leakproof, labeled or color-coded container that is closed before being removed from the work area.

(e)(2)(ii)(C) Access to the work area shall be limited to authorized persons. Written policies and procedures shall be established whereby only persons who have been advised of the potential biohazard, who meet any specific entry requirements, and who comply with all entry and exit procedures shall be allowed to enter the work areas and animal rooms.

(e)(2)(ii)(D) When other potentially infectious materials or infected animals are present in the work area or containment module, a hazard warning sign incorporating the universal biohazard symbol shall be posted on all access doors. The hazard warning sign shall comply with paragraph (g)(1)(ii) of this standard.

(e)(2)(ii)(E) All activities involving other potentially infectious materials shall be conducted in biological safety cabinets or other physical-containment devices within the containment module. No work with these other potentially infectious materials shall be conducted on the open bench.

(e)(2)(ii)(F) Laboratory coats, gowns, smocks, uniforms, or other appropriate protective clothing shall be used in the work area and animal rooms. Protective clothing shall not be worn outside of the work area and shall be decontaminated before being laundered.

1910.1030(e)(2)(ii)(G)

(e)(2)(ii)(G) Special care shall be taken to avoid skin contact with other potentially infectious materials. Gloves shall be worn when handling infected animals and when making hand contact with other potentially infectious materials is unavoidable.

(e)(2)(ii)(H) Before disposal all waste from work areas and from animal rooms shall either be incinerated or decontaminated by a method such as autoclaving known to effectively destroy bloodborne pathogens.

(e)(2)(ii)(I) Vacuum lines shall be protected with liquid disinfectant traps and high-efficiency particulate air (HEPA) filters or filters of equivalent or superior efficiency and which are checked routinely and maintained or replaced as necessary.

(e)(2)(ii)(J) Hypodermic needles and syringes shall be used only for parenteral injection and aspiration of fluids from laboratory animals and diaphragm bottles. Only needle-locking syringes or disposable syringe-needle units (i.e., the needle is integral to the syringe) shall be used for the injection or aspiration of other potentially infectious materials. Extreme caution shall be used when handling needles and syringes. A needle shall not be bent, sheared, replaced in the sheath or guard, or removed from the syringe following use. The needle and syringe shall be promptly placed in a puncture-resistant container and autoclaved or decontaminated before reuse or disposal.

(e)(2)(ii)(K) All spills shall be immediately contained and cleaned up by appropriate professional staff or others properly trained and equipped to work with potentially concentrated infectious materials.

1910.1030(e)(2)(ii)(L)

(e)(2)(ii)(L) A spill or accident that results in an exposure incident shall be immediately reported to the laboratory director or other responsible person.

(e)(2)(ii)(M) A biosafety manual shall be prepared or adopted and periodically reviewed and updated at least annually or more often if necessary. Personnel shall be advised of potential hazards, shall be required to read instructions on practices and procedures, and shall be required to follow them.

(e)(2)(iii) Containment Equipment.

(e)(2)(iii)(A) Certified biological safety cabinets (Class I, II, or III) or other appropriate combinations of personal protection or physical containment devices, such as special protective clothing, respirators, centrifuge safety cups, sealed centrifuge rotors, and containment caging for animals, shall be used for all activities with other potentially infectious materials that pose a threat of exposure to droplets, splashes, spills, or aerosols.

(e)(2)(iii)(B) Biological safety cabinets shall be certified when installed, whenever they are moved and at least annually.

(e)(3) HIV and Hep B Vaccination research laboratories shall meet the following criteria:

1910.1030(e)(3)(i)

(e)(3)(i) Each laboratory shall contain a facility for hand washing and an eye wash facility which is readily available within the work area.

(e)(3)(ii) An autoclave for decontamination of regulated waste shall be available.

(e)(4) HIV and Hep B Vaccination production facilities shall meet the following criteria:

(e)(4)(i) The work areas shall be separated from areas that are open to unrestricted traffic flow within the building. Passage through two sets of doors shall be the basic requirement for entry into the work area from access corridors or other contiguous areas. Physical separation of the high-containment work area from access corridors or other areas or activities may also be provided by a double-doored clothes-change room (showers may be included), airlock, or other access facility that requires passing through two sets of doors before entering the work area.

(e)(4)(ii) The surfaces of doors, walls, floors and ceilings in the work area shall be water resistant so that they can be easily cleaned. Penetrations in these surfaces shall be sealed or capable of being sealed to facilitate decontamination.

1910.1030(e)(4)(iii)

(e)(4)(iii) Each work area shall contain a sink for washing hands and a readily available eye wash facility. The sink shall be foot, elbow, or automatically operated and shall be located near the exit door of the work area.

(e)(4)(iv) Access doors to the work area or containment module shall be self-closing.

(e)(4)(v) An autoclave for decontamination of regulated waste shall be available within or as near as possible to the work area.

(e)(4)(vi) A ducted exhaust-air ventilation system shall be provided. This system shall create directional airflow that draws air into the work area through the entry area. The exhaust air shall not be recirculated to any other area of the building, shall be discharged to the outside, and shall be dispersed away from occupied areas and air intakes. The proper direction of the airflow shall be verified (i.e., into the work area).

(e)(5) Training Requirements. Additional training requirements for employees in HIV and Hep B Vaccination research laboratories and HIV and Hep B Vaccination production facilities are specified in paragraph (g)(2)(ix).

(f) HBV Vaccination and Post-exposure Evaluation and Follow-up --

1910.1030(f)(1)

(f)(1)General.

(f)(1)(i) The employer shall make available the hepatitis B vaccine and vaccination series to all employees who have occupational exposure, and post-exposure evaluation and follow-up to all employees who have had an exposure incident.

(f)(1)(ii) The employer shall ensure that all medical evaluations and procedures including the hepatitis B vaccine and vaccination series and post-exposure evaluation and follow-up, including prophylaxis, are:

(f)(1)(ii)(A) Made available at no cost to the employee;

(f)(1)(ii)(B) Made available to the employee at a reasonable time and place;

(f)(1)(ii)(C) Performed by or under the supervision of a licensed physician or by or under the supervision of another licensed healthcare professional; and(f)(1)(ii)(D) Provided according to recommendations of the U.S. Public Health Service current at the time these evaluations and procedures take place, except as specified by this paragraph (f).

(f)(1)(iii) The employer shall ensure that all laboratory tests are conducted by an accredited laboratory at no cost to the employee.

1910.1030(f)(2)

(f)(2) HBV Vaccination.

(f)(2)(i) HBV vaccination shall be made available after the employee has received the training required in paragraph (g)(2)(vii)(I) and within 10 working days of initial assignment to all employees who have occupational exposure unless the employee has previously received the complete hepatitis B vaccination series, antibody testing has revealed that the employee is immune, or the vaccine is contraindicated for medical reasons.

(f)(2)(ii) The employer shall not make participation in a prescreening program a prerequisite for receiving hepatitis B vaccination.

(f)(2)(iii) If the employee initially declines hepatitis B vaccination but at a later date while still covered under the standard decides to accept the vaccination, the employer shall make available hepatitis B vaccination at that time.

(f)(2)(iv) The employer shall assure that employees who decline to accept hepatitis B vaccination offered by the employer sign the statement in Appendix A.

(f)(2)(v) If routine booster dose(s) of hepatitis B vaccine is recommended by the U.S. Public Health Service at a future date, such booster dose(s) shall be made available in accordance with section (f)(1)(ii).

(f)(3) Post-exposure Evaluation and Follow-up. Following a report of an exposure incident, the employer shall make immediately available to the exposed employee a confidential medical evaluation and follow-up, including at least the following elements:

(f)(3)(i) Documentation of the route(s) of exposure, and the circumstances under which the exposure incident occurred;

1910.1030(f)(3)(ii)

(f)(3)(ii) Identification and documentation of the source individual, unless the employer can establish that identification is infeasible or prohibited by state or local law;

(f)(3)(ii)(A) The source individual's blood shall be tested as soon as feasible and after consent is obtained in order to determine Hep B Vaccination and HIV infectivity. If consent is not obtained, the employer shall establish that legally required consent cannot be obtained. When the source individual's consent is not required by law, the source individual's blood, if available, shall be tested and the results documented.

(f)(3)(ii)(B) When the source individual is already known to be infected with Hep B Vaccination or HIV, testing for the source individual's known Hep B Vaccination or HIV status need not be repeated.

(f)(3)(ii)(C) Results of the source individual's testing shall be made available to the exposed employee, and the employee shall be informed of applicable laws and regulations concerning disclosure of the identity and infectious status of the source individual.
(f)(3)(iii) Collection and testing of blood for Hep B Vaccination and HIV serological status;

(f)(3)(iii)(A) The exposed employee's blood shall be collected as soon as feasible and tested after consent is obtained.

1910.1030(f)(3)(iii)(B)

(f)(3)(iii)(B) If the employee consents to baseline blood collection, but does not give consent at that time for HIV serologic testing, the sample shall be preserved for at least 90 days. If, within 90 days of the exposure incident, the employee elects to have the baseline sample tested, such testing shall be done as soon as feasible.

(f)(3)(iv) Post-exposure prophylaxis, when medically indicated, as recommended by the U.S. Public Health Service;

(f)(3)(v) Counseling; and

(f)(3)(vi) Evaluation of reported illnesses.

(f)(4) Information Provided to the Healthcare Professional.

(f)(4)(i) The employer shall ensure that the healthcare professional responsible for the employee's HBVvaccination is provided a copy of this regulation.

(f)(4)(ii) The employer shall ensure that the healthcare professional evaluating an employee after an exposure incident is provided the following information:

(f)(4)(ii)(A) A copy of this regulation;

(f)(4)(ii)(B) A description of the exposed employee's duties as they relate to the exposure incident;

(f)(4)(ii)(C) Documentation of the route(s) of exposure and circumstances under which exposure occurred;

1910.1030(f)(4)(ii)(D)

(f)(4)(ii)(D) Results of the source individual's blood testing, if available; and

(f)(4)(ii)(E) All medical records relevant to the appropriate treatment of the employee including vaccination status which are the employer's responsibility to maintain.

(f)(5) Healthcare Professional's Written Opinion. The employer shall obtain and provide the employee with a copy of the evaluating healthcare professional's written opinion within 15 days of the completion of the evaluation.

(f)(5)(i) The healthcare professional's written opinion for HBVvaccination shall be limited to whether HBVvaccination is indicated for an employee, and if the employee has received such vaccination.

(f)(5)(ii) The healthcare professional's written opinion for post-exposure evaluation and follow-up shall be limited to the following information:

(f)(5)(ii)(A) That the employee has been informed of the results of the evaluation; and

(f)(5)(ii)(B) That the employee has been told about any medical conditions resulting from exposure to blood or other potentially infectious materials which require further evaluation or treatment.

1910.1030(f)(5)(iii)

(f)(5)(iii) All other findings or diagnoses shall remain confidential and shall not be included in the written report.

(f)(6) Medical Recordkeeping. Medical records required by this standard shall be maintained in accordance with paragraph (h)(1) of this section.

(g) Communication of Hazards to Employees --

(g)(1) Labels and Signs --

(g)(1)(i) Labels.

(g)(1)(i)(A) Warning labels shall be affixed to containers of regulated waste, refrigerators and freezers containing blood or other potentially infectious material; and other containers used to store, transport or ship blood or other potentially infectious materials, except as provided in paragraph (g)(1)(i)(E), (F) and (G).

(g)(1)(i)(B) Labels required by this section shall include the following legend:



(g)(1)(i)(C) These labels shall be fluorescent orange or orange-red or predominantly so, with lettering and symbols in a contrasting color.

(g)(1)(i)(D) Labels shall be affixed as close as feasible to the container by string, wire, adhesive, or other method that prevents their loss or unintentional removal.

1910.1030(g)(1)(i)(E)

(g)(1)(i)(E) Red bags or red containers may be substituted for labels.

(g)(1)(i)(F) Containers of blood, blood components, or blood products that are labeled as to their contents and have been released for transfusion or other clinical use are exempted from the labeling requirements of paragraph (g).

(g)(1)(i)(G) Individual containers of blood or other potentially infectious materials that are placed in a labeled container during storage, transport, shipment or disposal are exempted from the labeling requirement.

(g)(1)(i)(H) Labels required for contaminated equipment shall be in accordance with this paragraph and shall also state which portions of the equipment remain contaminated.

(g)(1)(i)(I) Regulated waste that has been decontaminated need not be labeled or color-coded.

(g)(1)(ii) Signs.

(g)(1)(ii)(A) The employer shall post signs at the entrance to work areas specified in paragraph (e), HIV and Hep B Vaccination Research Laboratory and Production Facilities, which shall bear the following legend:



(Name of the Infectious Agent)

(Special requirements for entering the area)

(Name, telephone number of the laboratory director or other responsible person.)

1910.1030(g)(1)(ii)(B)

(g)(1)(ii)(B) These signs shall be fluorescent orange-red or predominantly so, with lettering and symbols in a contrasting color.

(g)(2) Information and Training.

(g)(2)(i) Employers shall ensure that all employees with occupational exposure participate in a training program which must be provided at no cost to the employee and during working hours.

(g)(2)(ii) Training shall be provided as follows:

(g)(2)(ii)(A) At the time of initial assignment to tasks where occupational exposure may take place;

(g)(2)(ii)(B) Within 90 days after the effective date of the standard; and

(g)(2)(ii)(C) At least annually thereafter.

(g)(2)(iii) For employees who have received training on bloodborne pathogens in the year preceding the effective date of the standard, only training with respect to the provisions of the standard which were not included need be provided.

(g)(2)(iv) Annual training for all employees shall be provided within one year of their previous training.

1910.1030(g)(2)(v)

(g)(2)(v) Employers shall provide additional training when changes such as modification of tasks or procedures or institution of new tasks or procedures affect the employee's occupational exposure. The additional training may be limited to addressing the new exposures created.

(g)(2)(vi) Material appropriate in content and vocabulary to educational level, literacy, and language of employees shall be used.

(g)(2)(vii) The training program shall contain at a minimum the following elements:

(g)(2)(vii)(A) An accessible copy of the regulatory text of this standard and an explanation of its contents;

(g)(2)(vii)(B) A general explanation of the epidemiology and symptoms of bloodborne diseases;

(g)(2)(vii)(C) An explanation of the modes of transmission of bloodborne pathogens;

(g)(2)(vii)(D) An explanation of the employer's exposure control plan and the means by which the employee can obtain a copy of the written plan;

(g)(2)(vii)(E) An explanation of the appropriate methods for recognizing tasks and other activities that may involve exposure to blood and other potentially infectious materials;

1910.1030(g)(2)(vii)(F)

(g)(2)(vii)(F) An explanation of the use and limitations of methods that will prevent or reduce exposure including appropriate engineering controls, work practices, and personal protective equipment;

(g)(2)(vii)(G) Information on the types, proper use, location, removal, handling, decontamination and disposal of personal protective equipment;

(g)(2)(vii)(H) An explanation of the basis for selection of personal protective equipment;

(g)(2)(vii)(I) Information on the hepatitis B vaccine, including information on its efficacy, safety, method of administration, the benefits of being vaccinated, and that the vaccine and vaccination will be offered free of charge;

(g)(2)(vii)(J) Information on the appropriate actions to take and persons to contact in an emergency involving blood or other potentially infectious materials;

(g)(2)(vii)(K) An explanation of the procedure to follow if an exposure incident occurs, including the method of reporting the incident and the medical follow-up that will be made available;

(g)(2)(vii)(L) Information on the post-exposure evaluation and follow-up that the employer is required to provide for the employee following an exposure incident;

1910.1030(g)(2)(vii)(M)

(g)(2)(vii)(M) An explanation of the signs and labels and/or color coding required by paragraph (g)(1); and

(g)(2)(vii)(N) An opportunity for interactive questions and answers with the person conducting the training session.

(g)(2)(viii) The person conducting the training shall be knowledgeable in the subject matter covered by the elements contained in the training program as it relates to the workplace that the training will address.

(g)(2)(ix) Additional Initial Training for Employees in HIV and Hep B Vaccination Laboratories and Production Facilities. Employees in HIV or Hep B Vaccination research laboratories and HIV or Hep B Vaccination production facilities shall receive the following initial training in addition to the above training requirements.

(g)(2)(ix)(A) The employer shall assure that employees demonstrate proficiency in standard microbiological practices and techniques and in the practices and operations specific to the facility before being allowed to work with HIV or Hep B Vaccination.

(g)(2)(ix)(B) The employer shall assure that employees have prior experience in the handling of human pathogens or tissue cultures before working with HIV or Hep B Vaccination.

1910.1030(g)(2)(ix)(C)(g)(2)(ix)(C) The employer shall provide a training program to employees who have no prior experience in handling human pathogens. Initial work activities shall not include the handling of infectious agents. A progression of work activities shall be assigned as techniques are learned and proficiency is developed. The employer shall assure that employees participate in work activities involving infectious agents only after proficiency has been demonstrated.

(h)Recordkeeping --

(h)(1)Medical Records.

(h)(1)(i) The employer shall establish and maintain an accurate record for each employee with occupational exposure, in accordance with 29 CFR 1910.1020.

(h)(1)(ii) This record shall include:

(h)(1)(ii)(A) The name and social security number of the employee;

(h)(1)(ii)(B) A copy of the employee's hepatitis B vaccination status including the dates of all the hepatitis B vaccinations and any medical records relative to the employee's ability to receive vaccination as required by paragraph (f)(2);

(h)(1)(ii)(C) A copy of all results of examinations, medical testing, and follow-up procedures as required by paragraph (f)(3);

(h)(1)(ii)(D) The employer's copy of the healthcare professional's written opinion as required by paragraph (f)(5); and

1910.1030(h)(1)(ii)(E)

(h)(1)(ii)(E) A copy of the information provided to the healthcare professional as required by paragraphs (f)(4)(ii)(B)(C) and (D).

(h)(1)(iii) Confidentiality. The employer shall ensure that employee medical records required by paragraph (h)(1) are:

(h)(1)(iii)(A) Kept confidential; and

(h)(1)(iii)(B) Not disclosed or reported without the employee's express written consent to any person within or outside the workplace except as required by this section or as may be required by law.

(h)(1)(iv) The employer shall maintain the records required by paragraph (h) for at least the duration of employment plus 30 years in accordance with 29 CFR 1910.1020.

(h)(2) Training Records.

(h)(2)(i) Training records shall include the following information:

(h)(2)(i)(A) The dates of the training sessions;

(h)(2)(i)(B) The contents or a summary of the training sessions;

(h)(2)(i)(C) The names and qualifications of persons conducting the training; and

1910.1030(h)(2)(i)(D)

(h)(2)(i)(D) The names and job titles of all persons attending the training sessions.

(h)(2)(ii) Training records shall be maintained for 3 years from the date on which the training occurred.

(h)(3) Availability.

(h)(3)(i) The employer shall ensure that all records required to be maintained by this section shall be made available upon request to the Assistant Secretary and the Director for examination and copying.

(h)(3)(ii) Employee training records required by this paragraph shall be provided upon request for examination and copying to employees, to employee representatives, to the Director, and to the Assistant Secretary.

(h)(3)(iii) Employee medical records required by this paragraph shall be provided upon request for examination and copying to the subject employee, to anyone having written consent of the subject employee, to the Director, and to the Assistant Secretary in accordance with 29 CFR 1910.1020.

1910.1030(h)(4)

(h)(4) Transfer of Records.

(h)(4)(i) The employer shall comply with the requirements involving transfer of records set forth in 29 CFR 1910.1020(h).

(h)(4)(ii) If the employer ceases to do business and there is no successor employer to receive and retain the records for the prescribed period, the employer shall notify the Director, at least three months prior to their disposal and transmit them to the Director, if required by the Director to do so, within that three month period.

(h)(5) Sharps injury log.

(h)(5)(i) The employer shall establish and maintain a sharps injury log for the recording of percutaneous injuries from contaminated sharps. The information in the sharps injury log shall be recorded and maintained in such manner as to protect the confidentiality of the injured employee. The sharps injury log shall contain, at a minimum:

(h)(5)(i)(A) The type and brand of device involved in the incident,

(h)(5)(i)(B) The department or work area where the exposure incident occurred, and

(h)(5)(i)(C) An explanation of how the incident occurred.

(h)(5)(ii) The requirement to establish and maintain a sharps injury log shall apply to any employer who is required to maintain a log of occupational injuries and illnesses under 29 CFR 1904.

(h)(5)(iii) The sharps injury log shall be maintained for the period required by 29 CFR 1904.6.

(i)Dates --

(i)(1) Effective Date. The standard shall become effective on March 6, 1992.